

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Ignatius P. Delacroix DOB: 1958-09-30 (age 68) Member ID: MBR-49927

Sex: M

Requesting Provider

Practice: Westfield Cardiac Rhythm Center

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-11-12

Level of care requested: INPATIENT

Made-up data for a software prototype. Not a real patient.

Cardiology consultation (dictated)

WESTFIELD CARDIAC RHYTHM CENTER

Consultation - dictated, transcribed 09/18/2026

RE: Delacroix, Ignatius P. DOB 09/30/1958

Mr. Delacroix is a sixty-seven-year-old gentleman, seen today at the request of Dr. Haugen for consideration of a defibrillator. He had an inferolateral myocardial infarction in March of 2020 treated with a stent to the circumflex. He has not had any further cardiac events or procedures since that time.

His echocardiogram from earlier this month shows an ejection fraction of twenty-seven percent, which is not much different from last year.

Functionally, he tells me he gets short of breath after one flight of stairs and has to pause when walking his dog more than a block or so. He has occasional exertional chest tightness, Canadian Cardiovascular Society class II, relieved by rest. No rest symptoms. No syncope.

IMPRESSION: Ischemic cardiomyopathy, ACC/AHA stage C heart failure with reduced ejection fraction. Stable angina CCS class II. He has been on guideline-directed therapy for several years.

RECOMMENDATION: Primary prevention ICD is reasonable. He will meet with our nurse practitioner for a formal shared decision-making visit.

Dictated by Siobhan Mercer-Aho, MD. D: 09/17/2026 T: 09/18/2026

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Echocardiogram report

WESTFIELD ECHO LAB

DATE: 2026-09-08

NAME: DELACROIX, IGNATIUS

2D/Doppler TTE

LV EF (biplane MOD): 0.27

LV EDV 205 mL / ESV 150 mL

WMA: inferolateral AK, inferior HK

RV TAPSE 17 mm

MR mild

E/e' 16

SUMMARY: Severe LV systolic dysfunction (EF 0.27). Prior 08/2025 EF 0.29.

MD: Haruki Bellweather

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Six-minute walk test

Cardiac Rehab / Exercise Lab Six-Minute Walk Test

Date: 09/18/2026 Pt: Delacroix, I.

Distance: 310 m (predicted 520 m)

Borg dyspnea at end: 5/10. Borg fatigue 4/10.

SpO2 96% -> 94%. HR 68 -> 101.

Stopped once at 4 min for 20 s (dyspnea).

Tech: L. Faraday, EP

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Medication list

Medication List - Delacroix, Ignatius P. - printed 09/18/2026

1. Bisoprolol 10 mg daily - since 04/2020
2. Sacubitril/valsartan 97/103 mg BID - since 01/2022 (previously ramipril 04/2020-01/2022)
3. Spironolactone 25 mg daily - since 04/2020
4. Empagliflozin 10 mg daily - since 06/2022
5. Clopidogrel 75 mg daily - since 03/2020
6. Rosuvastatin 20 mg daily
7. Isosorbide mononitrate 30 mg daily
8. Nitroglycerin 0.4 mg SL PRN
9. Allopurinol 100 mg daily

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Shared decision making note

WESTFIELD CARDIAC RHYTHM CENTER

Shared Decision Making Visit - ICD Date: 09/25/2026

Patient: Ignatius Delacroix With: partner (Rhys)

NP: Galina Petrosyan, FNP-C

Decision aid: evidence-based ICD decision aid, reviewed in its entirety (paper).

Content covered: what an ICD is, likely benefit in his situation, shocks, infection, lead problems, battery replacement, turning the device off later in life, choosing no ICD.

Pt summary in own words: 'it's a safety net, not a cure.'

Choice: ICD.

G. Petrosyan FNP-C

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Laboratory results

WESTFIELD REGIONAL LAB - 09/20/2026

DELACROIX, IGNATIUS

Na 141, K 4.3, BUN 30, Cr 1.7 (H), eGFR 43

Uric acid 6.2

Hgb 13.8, Plt 199

BNP 488 (H)

INR 1.0

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Surgeon's admission plan

PRE-ADMISSION PLAN - ICD

Date: 10/01/2026

Patient: Delacroix, Ignatius P. / Admit 11/12/2026 inpatient / CPT 33249

Operator: S. Mercer-Aho MD

We anticipate two midnights in hospital after implant.

Supporting factors: CKD stage 3b (eGFR 43) w/ contrast venography; clopidogrel continued uninterrupted (pocket hematoma surveillance); gout w/ recent flare risk w/ diuretic adjustment.

Screen for limiting conditions: no cognitive impairment, no terminal illness, no atrial fibrillation, clinically stable. None present.

S. Mercer-Aho MD

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